Fax cover sheet

Mesa Verde Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Ruben Castellanos, MSW
Phone: 555-0160
Date: 07/17/2026 08:30
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

63F CAP + COPD, improving, completing PO abx.
Target d/c 07/17 PM once home O2 is set up. H&P, progress notes, RT and PT notes attached.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Mesa Verde Medical Center (fictional)
Dept/Floor: 4 North Medical
SW/CM/RN: Ruben Castellanos, MSW
Phone/Pager #: 555-0160
Email: rcastellanos@mesaverde.example
Nursing Station #: 555-0161
Authorized by: Dr. J. Whitcombe
Phone #:

Patient Information

Patient Name: Linda Doe MRN: MV-1186430 Date of Birth (dd/mm/yyy): 09/27/1962
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: Asian
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Community-acquired pneumonia, COPD
Admit Date: 07/11/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 07/11/2026 Results: CXR RLL infiltrate, improving

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* X No	Active Infection	Yes* X No
*Explain		*Explain	
Self Administer All Meds	X Yes No*	Ambulatory	X Yes No
Continent of Bowel & Bladder	X Yes No	Assistive Device	Yes* X No
Colostomy/Ileostomy	Yes X No	*Explain	
Foley Catheter	Yes X No	Hospice	Yes* X No
Diabetes	Yes X No	*Explain	
Insulin	Yes X No	Car	Yes X No
Diabetes related Oral Meds	Yes X No	Service Animal	Yes X No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Linda Doe

DOB: 09/27/1962

Member ID: OCH-40927315

Referring Provider

Facility: Mesa Verde Medical Center (fictional)

Clinical

Primary Diagnosis: Pneumonia, unspecified organism

ICD-10: J18.9

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 07/17/2026

Requested Days: 14

Secondary Dx: COPD with acute lower respiratory infection (J44.0)

Plan Use Only

Status: Pending review

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Patient face sheet

Mesa Verde Medical Center (fictional) - PATIENT FACE SHEET

Patient: Linda Doe
MRN: MV-1186430
Account #: 99186430
DOB: 09/27/1962
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 07/11/2026 22:10
Admit Type: Inpatient
Unit/Bed: 4N-409
Attending: Julian Whitcombe, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 07/11/2026

CC

Fever, productive cough, worse SOB x 4 days

HPI

63 y/o F with COPD (GOLD 2 per 2024 PFTs), current smoker 1/2 ppd, homeless sleeping in her tent near Example Park x 2 years, presents with 4 days of fever, rusty sputum and dyspnea. No home O2 at baseline. EMS SpO2 85% RA. Uses albuterol inhaler PRN, ran out 1 month ago.

PMH

- COPD
- GERD

Meds PTA

- Albuterol HFA PRN (out x 1 month)
- Omeprazole 20 mg daily

Allergies

NKDA

Social

Homeless, tent. Smokes 1/2 ppd. EtOH denies. Drugs denies.

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History and Physical - page 2

Exam

T 101.6F HR 114 BP 118/70 RR 26 SpO2 86% RA -> 93% on 3L NC
Gen: ill-appearing, speaking full sentences
Lungs: RLL crackles and bronchial breath sounds, scattered expiratory wheeze
CV: tachy, regular
Neuro: AOx4

Data

WBC 17.9, lactate 1.6, procalcitonin 1.8
CXR: RLL consolidation, no effusion, no pneumothorax
COVID/flu/RSV PCR negative

Assessment/Plan

63F with CAP RLL and COPD with acute exacerbation.
1. CAP - ceftriaxone 1 g IV daily + azithromycin 500 mg IV daily; blood cx x2, sputum cx
2. AECOPD - duonebs q4h, prednisone 40 mg PO daily x 5 days. Titrate O2 to SpO2 88-92%. No BiPAP needed.
3. Tobacco - nicotine patch 14 mg, counseling
4. Dispo - RT, PT; CM for recuperative care
Code: Full. DVT ppx: heparin 5000 units SC q8h
Signed: Julian Whitcombe, MD 07/11/2026 23:40

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#6

Date: 07/16/2026 07:50
S: Cough improving, sputum now clear. No fevers x 72h. Walking in hall with RT.
O: T 98.5F HR 88 BP 122/74 RR 18 SpO2 91% RA at rest; 86% RA with ambulation -> 92% on 2L NC
Lungs: faint RLL crackles, minimal wheeze
Blood cx 07/11 x2: no growth, final. Sputum cx: normal flora.
A/P:
- CAP RLL improving. Transitioned 07/14 to cefpodoxime 200 mg PO BID to complete 7 days total (last dose 07/17 PM). Azithromycin course complete 07/15.
- COPD - prednisone course complete 07/15. Tiotropium + albuterol MDI.
- New exertional and nocturnal hypoxemia: qualifies for home O2 (see RT). Order home oxygen 2 L/min NC with exertion and sleep.
- Medically stable for d/c 07/17 once home O2 arranged
Signed: Julian Whitcombe, MD

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Respiratory therapy note

RESPIRATORY THERAPY NOTE

Date: 07/16/2026 09:30

Breath sounds: diminished RLL, scattered end-exp wheeze, improved from admission

Home O2 Qualification Walk

SpO2 RA at rest: 91%

SpO2 RA with ambulation 200 ft: nadir 86%

SpO2 on 2L NC with ambulation: 92-93%

Overnight oximetry 07/15: SpO2 <88% for 41 min on RA

Meets criteria for home O2 with exertion and sleep. Not required at rest.

Treatments: duonebs stopped 07/15. Tiotropium 18 mcg daily + albuterol MDI PRN with spacer, technique correct on teach-back.

Pt educated on O2 safety, no smoking near oxygen; verbalized understanding.

Signed: A. Bernal, RRT

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Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 07/15/2026 11:00

Dx: CAP, COPD, deconditioning

Prior Level of Function

Independent ambulation without device.

Assessment

Bed mobility: Independent

Sit<>stand: Independent

Transfers: Independent

Gait: 200 ft without device, modified independent, 1 standing rest break, on 2L NC portable O2

Stairs: not tested

Balance: static good, dynamic good

Plan / Recommendation

Safe for d/c to recuperative care. Will need portable O2 for walking. No DME needs other than O2.

Signed: K. Ostrowski, PT

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Case management note

CASE MANAGEMENT NOTE

Date: 07/16/2026 10:40
Pt AOx4, agreeable to recuperative care. Independent with ADLs per nursing.
Home O2 order (2 L/min NC with exertion and sleep, concentrator + portable tanks) faxed to Pacific Breeze Medical Supply (fictional), 555-0167, on 07/16/2026 10:15.
Awaiting vendor confirmation of delivery date and delivery address.
PCP follow-up: not yet scheduled; pt to establish with clinic after d/c.
Signed: Ruben Castellanos, MSW

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Vitals and labs

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
07/15 08:00	98.6	92	124/76	18	91%	RA
07/15 16:00	98.4	90	120/72	18	92%	RA
07/16 00:00	98.1	84	118/70	16	93%	2L NC (sleep)
07/16 07:50	98.5	88	122/74	18	91%	RA
07/16 12:00	98.3	86	120/74	16	92%	RA

LABS

07/11 | WBC 17.9 | Hgb 13.2 | Plt 342 | Cr 0.8 | Procalcitonin 1.8
07/15 | WBC 10.6 | Hgb 12.7 | Na 138 | K 4.2 | Cr 0.7 | Glu 104

MICROBIOLOGY

07/11 Blood cx x2: No growth, final
07/12 Sputum cx: Normal respiratory flora
07/12 MRSA nares PCR: Negative
07/17 05:30 SARS-CoV-2 rapid antigen: Negative (added 07/17 AM before d/c)

IMAGING

07/11 CXR: RLL consolidation
07/15 CXR: improving RLL opacity, no effusion or pneumothorax

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Discharge medication list

DISCHARGE MEDICATION LIST

Patient: Linda Doe MRN: MV-1186430

- Cefpodoxime 200 mg PO BID - last dose 07/17 PM
- Tiotropium 18 mcg inhaled daily (NEW)
- Albuterol HFA 90 mcg 2 puffs q4-6h PRN SOB, with spacer
- Omeprazole 20 mg PO daily
- Nicotine patch 14 mg/24h daily

DME / Respiratory

- Home oxygen 2 L/min via nasal cannula with exertion and during sleep
- Spacer

Prednisone and azithromycin courses complete.

Patient able to self-administer all medications: Yes

30-day supply dispensed from outpatient pharmacy: Yes

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